thinking, that has come about in each country as the country achieved a greater intellectual capacity in public policy. We in India will need to achieve an improved intellectual capacity, in public policy work on public health and on healthcare, in order to obtain the full translation of higher GDP into improved health.

Summing up

Health policy is one of the hardest parts of public policy. The toolkit of market failure and state capacity gives us important insights into the field. There is a neat and vertical split between two classes of market failure (public goods and externalities) that shape the field of ‘public health’ or ‘common goods for health’ and the other two classes of market failure (asymmetric information and market power) that shape healthcare.

We in India need to change course. Under the present trajectory, GDP growth is inducing greater ill health through problems such as pollution, natural disasters and road safety. In the government, public health capabilities have atrophied: we do worse on critical problems like vector control when compared with many decades ago. Voters are unhappy, and the government is trying to subsidize their purchase of private healthcare services from a private healthcare system that suffers from extensive market failure. Thinking about market failure and state capacity from first principles is required in public health and in healthcare.

As in many other fields, there is a lot of interest in technological fixes for health policy. There is undoubtedly great opportunity in using better technology, ranging from mundane process automation using better data handling, all the way to artificial intelligence (AI) systems that will replace doctors. However, the foundation of beneficial technology adoption lies in incentives. Our first priority has to be to address the foundations: to reorganize health policy around the problems of market failure and state capacity.